



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه حسين سيد مرسى
المحافظة : القاهرة
الحى / المدينة : مصر القديمة
العنوان : -
رقم البطاقة : 25310170100721 رقم التليفون : 01009410211
تاريخ تسجيل الإستمارة : 26-09-2026
العمر (تاريخ الميلاد) : 17-10-1953 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المدونة فى هذا النموذج صحيحة.
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمه حسين سيد مريم
National ID: 25310170100721



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمة
حسين سيد مرسى
مساكن عين الصيرة ١٩٠٩ م
مصر القديمة - القاهرة

١٩٥٣/١/١٧ ٢٥٣ ١٠ ١٧ ٠١ ٠٠ ٧ ٢١
KZ0337895

DMS
مفكرة

٢٠٢٥/٠٢ ٢٥٣ ١٠ ١٧ ٠١ ٠٠ ٧ ٢١

ربة منزل
أنثى
البطاقة سارية حتى ٢٠٢٢/٠٢/٠١

١

Patient's Name: fatma hussin sayed morsey
Date: 2026-04-29
Referred by: Prof.Dr. NCAP
Hospital: Goustave Roussey International Egypt

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE NECK,
CHEST, ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA with no available previous films for comparison.

I- NECK:

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subcentimetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

II- CHEST:

- Porta cath is noted.
- Left breast lesion is seen measuring 59 x 58 mm and right breast calcifications for sono-mammography.
- Tiny right upper lobe calcified pulmonary nodule measuring 2.4 mm in diameter.
- Few bilateral scattered atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

III- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Average sized spleen showing multiple tiny calcific foci.

Patient's Name: fatma hussin sayed morsey
Date: 2026-04-29
Referred by: Prof.Dr. NCAP
Hospital: Goustave Roussey International Egypt

MDT FORM 2719 / 2025 G

Name	فاطمة حيت	MRN	C. 001005
Age	72	ID	5031-17-100-791
Menopausal status	Post-menopausal	Phone	1009210211
NRS SCORE		Marital status	Widow
BMI		Family history	the Aunt Breast.
CO morbidity	-ve	REASON OF MDT	Remitt

Clinical Data

BX: IDC

R

PS 1

L

ER: 0/8 PR: 0/8 HER 2: +3 (+3) Ki67:

Sonomammography: 3/9
PET/CT 2025.

Mets Workup: RT supra- and infra clavicular W.
largest 1.2 cm, submax 4.1
② RT breast retro-areolar Mass lesion
4.7 x 5.1 cm with submax 29.6

Recommendations: ③ Per. Vascular, Pre-tracheal, RT hilar, Cardio-Phrenic
subcarinal LN largest 1.3 cm, submax 6.1
④ Proximal sigmoid Colon with submax 11.5, with no definite
thickenings or masses, with low grade FDG Pericolic, H external
ileocolic LN, 1.2 cm sub 3 → total diverticulosis of desc. col.
⑤ gastro-hepatic, Porto-caval, Para-aortic, retro-caval, epidemic LN
and proximal sigmoid colon.

MDT OUTCOME & PLAN largest measures 11 cm, submax 5.3

Based on Metabolic Act Med LN 5.3 cm
col - total (26 lesions)

+ Colonoscopy

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأمل / فاطمة حسين محمد مرسي	Request No.	: 13007792
Patient No.	: 25310170100721	Request Date	: 06/03/2025
Path No.	: SR-376/25	Verification Date	: 24/03/2025
Sex	: Female	Financial Category	: أورام دثر السلام - نفقة الدولة
Age	: 71 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Slide Revision.

Gross Examination:
- One paraffin block labeled by (200497/25) and 3 slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2).
- * DCIS: Not detected.
- * Angioinvasion: Not detected.
- * Perineural invasion: Not detected.
- * Microcalcifications: Not detected.
- * IPT:
- ER: No nuclear reaction of tumor cells (score 0/8).
- PR: No nuclear reaction of tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:
Left breast mass. Slide revision:
- Invasive duct carcinoma, grade III.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

Verified by:
Dr. Toka Medhat

Printing Date : 22/05/2025 11:26:05

Page 1 of 2

Printing By : hermal

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATIENT NAME: FATMA HUSEN SAYED MORSY

GENDER: Female

DATE : 18-Feb-2025 09:25 AM

PATIENT ID: 20225016550

AGE: 71 y

ORDER: Z164906,Z164907,
Z164977**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND****INDICATION:**

- Left para-areolar palpable lump 3 months ago.

TECHNIQUE:

- CC and MLO view.
- Additional DBT.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).

Left breast:

- Left retro-areolar focal asymmetry by complementary DBT revealed two adjacent obscured high dense masses.
- Left few (about 3 masses) scattered oval, equal dense masses one of them is calcified. This was confirmed by Complementary Ultrasound :
- Left 12 o'clock para-areolar oval micro-lobulated hypoechioc vascular mass with cystic component showing anterior and posterior intraductal extensions, measuring about 4.2x 2.2x 4 cm
- with similar smaller adjacent mass seen 2x 1x 1.7 cm.
- Left few scattered oval circumscribed hypoechioc masses seen at the following:
 - *Left 2 o'clock zone b, two masses measured 1.8x 0.7 cm and 1.1 x 0.4 cm,
 - *Left 4 o'clock calcified mass 1.6x 1.4cm.
 - *left 7 o'clock 1x 0.5 cm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.
- No pathologically enlarged regional LNS

Right breast:

- Right UOQ, posterior third focal asymmetry corresponding to right 1/2 o'clock condensed glandular tissue with few adenotic nodules seen.
- Right UOQ and UIQ few scattered partially obscured masses some of them are calcified corresponding to by U/S:
- Left few scattered oval circumscribed hypoechioc masses seen at the following:
 - *right 8/9 o'clock measures 1.4x 0.5 cm..
 - *At 10 o'clock two masses one of them is calcified measures 1.1x 0.5 cm and 1.2x 0.8 cm.
 - *right 11 o'clock measures 1.2x 1 cm.
 - *retro-areolar 12 o'clock 1.8x 0.6 cm
 - 12 o'clock mid zone measures 2x 1.8 cm ..calcified.

PATIENT: FATMA HUSEN SAYED MORSY, 20225016550, Female, 71 y

Report version 2, 18-Feb-2025 11:13 AM



د. محمد
م. محمد

PATIENT NAME: FATMA HUSEN SAYED MORSEY

GENDER: Female

DATE : 03-Mar-2025 12:37 PM

ORDER: Z169394,Z169395

PATIENT ID: 20225016550

AGE: 71 y

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

FINDINGS:

1-CHEST

- Both breasts show multiple calcified small masses with left breast retroareolar mass is seen measuring about 1.7x1.7 cm and another mass is seen at the lower inner quadrant measuring about 1.1x1 cm.
- Prominent single left axillary level 1 lymph node measuring 1.2X0.6 cm.
- Multiple enlarged left supraclavicular lymph nodes, the largest measures 2X1 cm.
- Right axillary level 1 subcentimetric lymph nodes, the largest measures about 1x0.8 cm.
- Bilateral scattered multi-segmental millimetric pulmonary nodules, largest at left lower lung measuring 5 mm.
- Right middle and right lower superior lung lobes show atelectatic bands with mild bronchiectatic changes calcific nodules.
- Enlarged prevascular, right and left cardiophrenic lymph nodes, largest is the former measuring 1.7X0.9 cm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Hiatus hernia.

2-ABDOMEN AND PELVIS

- Average sized liver showing tiny segment VII focal lesion measuring 0.5 cm.
- Average sized spleen showing multiple calcific foci of post granulomatous infection.
- Enlarged right kidney showing renal pelvis stone measuring 1.6X2 cm surrounded by fat smudging, stranding, upper ureteric mural thickening with consequent ballooning of pelvi-calycal system and marked thinning out of the parenchyma. Another multiple calycal variable sized stones.

- Small sized left kidney showing parenchymal thinning out with multi-calyceal variable sized stones, the largest measures 2X1.5 cm.
- Multiple enlarged epigastric, peri-pancreatic and para-aortic lymph nodes, largest is latter measuring 1.8X1.2 cm.
- Normal appearance of the pancreas and adrenals organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- Lower descending and upper sigmoid colon diverticular disease with a segment of mural thickening of reaching 1.8 cm surrounded by mild fat smudging.
- Thickened hypodense endometrium measuring 1.9 cm.
- Two paraumbilical hernias, superior fat containing one with a defect of 2.5 cm and inferior fat and bowel containing with a defect measuring 4.7 cm.
- Bilateral inguinal fat containing hernia more evident at left.
- **The revised bone window revealed:**
 1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
 2. Right 10th rib posterior aspect sclerotic focus (likely bony island).
 3. Spondylotic changes.

Opinion:

A reported case of left breast cancer showing:

- Prominent left axillary level I lymph nodes.
- Left supra-clavicular, mediastinal prevascular and cardiphrenic as well as abdominal lymphadenopathy.
- Segment VII tiny hepatic focal lesion, for dynamic MRI study.
- Right renal pelvis stone with consequent ballooning of the pelvicalyceal system and parenchymal thinning out with sign of obstructive nephropathy.
- Small sized left kidney with multiple calyceal stones.
- Descending colon and sigmoid diverticular disease surrounding by mild inflammatory changes suggesting of diverticulitis.
- Thickened endometrium, for pelvic/transvaginal ultrasound.

Reported By,
Dr. Yomna Tarek, MSc
Dr. Reham Anwar, FEBR,MD

04-Mar-2025 12:05 PM

Patient Name : الأستاذة / فاطمة حسين سيد مرسى
Patient No. : 25310170100721
Path No. : SR-376/25
Sex : Female
Age : 71 Y
Referring Doc :

Request No. : 13007792
Request Date : 06/03/2025
Verification Date : 24/03/2025
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Slide Revision.

Gross Examination:

- One paraffin block labeled by (200497/25) and 3 slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
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- * DCIS: Not detected.
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- * Microcalcifications: Not detected.
- * IPT:
 - ER: No nuclear reaction of tumor cells (score 0/8).
 - PR: No nuclear reaction of tumor cells (score 0/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast mass, Slide revision:

- Invasive duct carcinoma, grade III.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

Verified by :

Printing Date : 19/05/2025 09:36:08

Page 1 of 2

Printing By : hermal



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Fatma Hussein Sayed Morsi	Date : 06/03/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 71 Year(s)
Medical ID: 742/2025	Pat. ID: 98251

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

Liver: -

- Average in size (about 16 cm at the right MCL), fine coarse echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- Average size , normal wall thickness, no stones.CBD is not dilated.
- **Spleen:** Average size (About 11 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site , normal echogenicity , bilateral renal stones largest right side about 1 cm , right mild to moderate aback pressure changes , no solid or cystic lesion. Right simple cysts
- Both ureters are noted dilated.

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -:

- anti-verted, average sized uterus. Endometrial thickness measures about: mm, no focal lesions.

Ovaries: - both are average in size with no dominant follicle could be detected.

- No free fluid in the Douglas pouch.
- Para-aortic and pancreatic area: no gross lesion

Opinion:

- bilateral renal stones largest right side, right mild to moderate aback pressure changes , Right simple cysts
- marked colonic distension .

Signature Dr. Sherien Ali El Sharkawy
PM 2:03 2025/03/06

Nuclear Medicine Unit

وحدة الطب النووي

Fatma Hussien Sayed

Date: 15/04/2025

Tc99m MDP SKELETAL SCINTIGRAPHY

History

- A 71-year-old patient with history of breast cancer.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No definite evidence of bone secondaries could be seen.
- Degenerative changes are noticed at different levels of spine and large joints.

IMPRESSION

- The skeleton is scintigraphically free from metastases.

Appreciated,

Dr. Hossam Sekkina, M.Sc.

Dr. Alaa Nagi, MD
Nuclear Medicine



25
عام
من
الخدمة

Hotline

07 21 29 2000

فرع حلوان : 64 شارع عبد الرحمن تقاطع شارع عبد الله فوق بنك مصر فرع نادي الاسمنت
فرع فيصل : 434 شارع الملك فيصل (فوق مستشفى تبارك للأطفال) الجيزة
فرع الدقي : 92 شارع التحرير - برج ساريدار الطبي - الدور الثالث
فرع شبرا : 128 شارع شبرا - أمام قسم الساحل - بجوار سنترال شبرا
فرع أكتوبر : رنا مول بعد الحصري، والتحصيد والتجارة، طبقه 3، مبنى 3، شبراخيت، أمانة
DOCTYPE BONE SCAN

FATMA HUSSEIN

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND
PELVIS REVEALED:**

I- CHEST:

- Suspected left breast retroareolar lesion measuring 4.1 x 3.3 cm for sono-mammography.
- Few bilateral breast calcified foci.
- Few bilateral scattered small pulmonary nodules largest measuring 5.4 mm at the left lower lobe.
- No pulmonary masses or cavitory lesions.
- No consolidation or collapse.
- Few prevascular lymph nodes largest measuring 14 mm in diameter.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing a small faintly enhancing right hepatic lobe hypodense focal lesion measuring 10 x 8 mm for dynamic MR study. No dilated biliary radicles.
- Right kidney is enlarged in size showing few stones within largest seen at the renal pelvis measuring 26 x 21 mm (staghorn stone). Evident fat stranding is noted around the pelvi-ureteric junction.
- The left kidney shows multiple stones largest measuring 20 x 13 mm at the lower calyx with no backpressure changes.
- Multiple para-aortic, bilateral iliac and gastro-esophageal junction lymph nodes largest measuring 16 mm at the para-aortic region.
- Diverticular disease of the sigmoid and descending colon.
- The spleen shows few tiny calcific foci ? post granulomatous sequelae.
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

- Two midline anterior abdominal wall hernias are noted containing bowel loops.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No ascites.
- Left inguinal hernia containing fat.
- ***For PET-CT assessment.***

III- Bone Survey:

- Two tiny L3 and L4 vertebral bodies lytic foci for bone scan to exclude metastasis.

Dr. Sherief Wagdi, MSc.

FATMA HUSSEIN

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

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- ***For PET-CT assessment.***

III- Bone Survey:

- Two tiny L3 and L4 vertebral bodies lytic foci for bone scan to exclude metastasis.

Dr. Sherief Wagdi, MSc.

Name : Fatima Hussein Said Morsi Date : 05-09-2026 01:03
Branch : Dokki Medical No. : 454827
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعافى مستشفى

Tc-99m MDP follow up SKELETAL SCINTIGRAPHY

History

73 year-old female patient, who gave history of treated left breast cancer and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- *Considering the last bone scan report dated 15/4/2025*, the current scan revealed the same scintigraphic features with no evidence of tracer uptake abnormalities that would account for bone secondaries.
- Degenerative features are depicted at different levels of the spine.
- Osteoarthritic changes are noticed around big joints.
- Tracer retention is noted in the right kidney, for more urological evaluation.

IMPRESSION:-

- The skeleton is keeping free from mctastasis .

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

Patient's Name:
Date:
Referred by:
Hospital:

Fatima Hussein Said
2026-08-13
Prof.Dr. no doctor
Goustave Roussey International Egypt

MULTISLICE NON CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of breast cancer noted bilateral breast masses. left breast well defined mass lesion large measuring (8.3x8.1cm) and right ill defined mass lesion measuring (6.6x3.6cm) both are showing macro calcifications along noted with right axillary subcentimetric lymph nodes with the largest measuring (0.9x0.5cm)
- Noted small calcified pulmonary nodule measuring 5x4mm seen at the right lung middle lobe just near the horizontal fissure with interfissural minimal pleural effusion.
- Clear Left lung field with No visualized nodules or consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial collection.
- Normal plain CT appearance of the heart and great vessels expect for Aortic & coronary atherosclerotic changes.
- Port A cath seen in place.
- Noted dorsal spine vertebral spondylosis.

II- ABDOMEN AND PELVIS:

- Average liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Bilateral Renal multiple Calculi, more than 5 stones in the right kidney , largest is pelvic measuring (2.5x1.8cm) & 4 in the left kidney largest is lower calyceal (2.2x2.0), with Right marked hydronephrotic changes and right renal cortical cyst measuring (7.7x5.4cm) .
- Normal CT appearance of the spleen, pancreas, suprarenal glands, and IVC.
- Normal aortic caliber with diffuse calcific atherosclerotic changes.

Patient's Name:
Date:
Referred by:
Hospital:

Fatima Hussein Said
2026-08-13
Prof.Dr. no doctor
Goustave Roussey International Egypt

- Normal CT appearance of pelvic organs with no pelvic masses or collections
- The urinary bladder shows no evidence of gross masses, diverticulae, or stones.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Noted Lumbar vertebral spondylosis.
- Noted umbilical hernia with intestinal & fatty content and supra umbilical mid line fatty omental hernia.

Opinion:

- Known case of breast cancer with bilateral breast masses for Sonomamography.
- Right middle lung lobe small calcified nodule with interfissural minimal pleural effusion.
- Bilateral renal Calculi with right marked hydronephrosis and right renal cyst.
- Noted Dorsolumbar vertebral spondylosis.
- Thoracic & abdominal Aortic Atherosclerosis.
- Anterior Abdominal wall Hernias.

Best regards,

Dr. Ayman Shawky, FEBR